## The Meaning of Normality

What we consider normal and what we consider abnormal, disturbed or psychopathological obviously has great import for clinical psychology and psychiatry. Yet, there is no clear consensus as to a definition of normality against which particular types of deviant or disordered behavior can be considered abnormal. Discussions of normality and abnormality, mental health and illness, are inevitably filled with value-laden terms (Jahoda, 1958; M. B. Smith, 1961). It seems impossible, nor do I believe it desirable, to avoid values in conceiving mental health. What is necessary is that the value framework be made explicit.

Offer and Sabshin (1966) have surveyed the many meanings of normality which have arisen in psychology, psychiatry, sociology, and anthropology. Building on their analysis let us consider alternate views of normality.

### NORMALITY AS HEALTH

In this meaning, common to both traditional medical-psychiatric and lay usage, normality simply means "not sick." If no gross pathology is present, behavior is said to be normal. Normality is then the wide range of functioning which has in common that it does not represent a disease state. In this context, Offer and Sabshin note, health refers to a *reasonable* rather than an *optimal* state. Reflected in clinical practice, this view would go along with efforts to reduce symptoms and restore adequate functioning. In research, this perspective provides a relatively straightforward criterion and avoids some of the difficulties of assessing degree of positive health. Thus, numerous studies have been made in which the behavior of clinical groups, defined by symptoms, hospitalization, or clinic attendance, are contrasted with "normals," persons who do not have symptoms and are not hospitalized or treated. Implicit in this view is the notion that the great mass of people are normal, while some few are abnormal.

### NORMALITY AS IDEAL (UTOPIA)

By contrast, a second perspective seeks a definition of normality in terms of a desirable or ideal state, which describes optimal rather than reasonable or average personality functioning. Views of *positive* mental health (e.g., Jahoda, 1958) or those which emphasize the goal of psychological development as self-actualization (Maslow, 1954; K. Goldstein, 1939), becoming a "fully functioning person" (Rogers, 1963), or attaining the "mature personality" (Allport, 1961) are of this

class. Such definitions of normality often emerge in discussions of the ideal goals of psychotherapy. This is in contrast to a view of treatment as removing symptoms and restoring comfort (achieving normality as health). In actuality, however, the ideal normal may be represented in few if any living mortals. It is in this sense that Freud once characterized normality as an ideal fiction. Clearly, the criteria by which an ideal state is to be defined are more difficult to establish and assess and more dependent on the values of the investigator than those which describe a not-sick or an average man. However, it is a necessary perspective, particularly as we are concerned with prevention of pathology and optimizing the personality functioning of large numbers of adequate but limited people.

### NORMALITY AS AVERAGE

A third perspective derives from the statistics of measurement. If any human attribute is measured, values fall along a "normal" or bell-shaped curve; most will cluster near the center and increasingly fewer cases will be found the farther out one goes. In many areas of psychological and medical measurement extensive normative data exist which define the average and describe the distribution of numerous characteristics, such as height, intelligence, blood pressure, reading skill, aggressiveness, and so on. Within a statistical conception, it should be noted that the average is normal, while *both* extremes are abnormal. A physician may speak of blood pressure as abnormally high or low; we commonly use the same phrase in describing, say, a person's height. It is worth noting, however, that although description along statistical lines is often described as scientific (i.e., value-free), where characteristics are clearly desirable, we rarely use the term abnormal to characterize one extreme. Thus, the person with an IQ of 150 is not likely to be called abnormally intelligent, nor is the perfect body described as abnormally beautiful. The statistical concept of normality has, nonetheless, considerable utility for describing the empirical distribution of human characteristics and is widely used in clinical research and practice.

### NORMALITY AS SOCIALLY ACCEPTABLE

A related view of normality is based on the premise that behavior can only be judged in terms of the social context within which it occurs. If it conforms with the normative expectations of the society, it is normal; if it does not, it is deviant or abnormal. Some social scientists have defended the view that what clinicians conceive as personality aberrations should be understood as social deviance and that universalistic definitions of health and illness are misguided. Ralph Linton (1956, p. 637) says "The tests of absolute normalcy are the individual's ability to apprehend reality, as understood by his society, to act in terms of this reality, and to be effectively shaped by his society during his developmental period. The test of relative normalcy is the extent to which the individual's experience has given him a personality conforming to the basic personality of his society." The *cultural relativism* of anthropologists in the 1930s was an extreme of this point of view (e.g., Benedict, 1934). The evidence they drew on was of three sorts: (1) There are some behaviors considered normal in our society which are abnormal in others. Thus, Benedict points out, the normally expected initiative and drive of the white American is looked on as severely disordered behavior among the Zuni Indians; (2) some behaviors considered abnormal in our society are normal else-

where. The northwest American Indians, Benedict reports, have institutionalized paranoid behavior; and (3) it is noted that there are forms of psychological disturbance found in other cultures which are not known in our own, such as going "amok" among the Malaysians. It is certainly true that even between subcultures of a complex society such as ours there are vast differences in social norms, which would make a particular behavior expectable and approved in one place, condemned in another. It is equally true that the clinician must know and interpret the social context of his patient if he is to understand his personality functioning and problems. But does this mean that there are no general culture-free criteria by which behavior can be defined as normal and abnormal? Critics of cultural relativism (e.g., Wegrocki, 1939) point out that too often the evidence for a cultural concept of abnormality rests on behavioral data alone and does not take into account the meaning of the behavior for the economy of the personality. A culturally determined act of another society may look like a psychopathological symptom of our own society, and vice versa, but on examination it is not accompanied by the anxiety, defensive properties, ego-alien, and compulsive experiences which are the proper marks of pathology.

One of the difficulties of conceiving normality from a cultural or statistical vantage point is the danger of identifying normality with conformity and conversely, abnormality with nonconformity, originality, or individuality. While it is true that the acts of mentally disturbed people often flout social norms, it is equally true that rigid adherence to norms can indicate pathology. Weak and uncertain people find security in being just like everyone else; to stand out in any respect is to be vulnerable.* Alberto Moravia's novel *The Conformist* graphically depicts the pathetic situation of a man who wants nothing more than to be average during the era of Italian Fascism, in the hope that he can thus erase the guilt of his childhood. By contrast, experimental studies have shown that leaders compared to more average men more often held fast to their points of view even when they seemed to be in disagreement with their group (Crutchfield, 1955). Erratic and unpredictable behavior can as well go with creativity as with mental illness. From the standpoint of psychological health, what matters is the motivation of the act rather than the degree to which it conforms to social expectation. Many psychotic individuals would want to conform, but they cannot comprehend the norm or they cannot control their actions sufficiently well to do so. Indeed, in small ways within their control, grossly disturbed people are often remarkable conformists, being eager not to offend. On the other hand, voluntary nonconformity—even to the extent of civil disobedience—may be the mark of a healthy personality, acting with conviction on his own values.

## NORMALITY AS PROCESS

Some theories attempt to characterize normality in terms of processes over time rather than in cross-sectional perspective. They emphasize the evolutional nature of biological or psychological systems, so that the valuation of a particular

* In this connection, I must express a strongly held prejudice against the use of the term "adjustment" in mental health. The image it brings to mind, mine at least, is of accommodation of the person to the social norm, with little concern for his individuality. When I hear the phrase, "a well-adjusted child," I have a slight chill, expecting to see a colorless youngster, harmless I suppose, who has been compressed in a common mold. If I use the word anywhere in this book, it will be an oversight.

behavior may alter depending on the phase of the life process. Thus, Erikson (1963) as earlier noted, views the epigenesis of personality development in terms of the successful mastery of successive stages of development, which leads to adult functioning and normality. It is the temporal process, rather than status at points along the way, that defines normality. For theoretical analysis, this distinction is important, though for our present survey process positions can be considered together with those that stress status.